

Honest Funeral

Where the Pain Is, and Where the Product Wins

Market, customer-pain, and channel research for a funeral-navigation product sold to hospices and provided free to patient families.

Prepared for Ryan Currie | July 2026 | ~60 primary and secondary sources; headline figures verified against primary documents (NFDA, MedPAC, FTC, CMS, Federal Register, DOJ).

Executive Summary

The U.S. funeral market is a textbook case of a broken consumer market: purchases average **\$8,000–\$13,000 all-in**, are made in **24–72 hours** by grieving buyers, from sellers whose prices for identical services vary **2–5x within the same city** and who mostly refuse to publish prices. A majority of families never contact a second funeral home. Roughly **4 to 6 in 10 Americans could not cover a funeral today without borrowing**.

Hospice is the single best channel to reach these families before the purchase locks in. About **52.9% of Medicare decedents** — more than **1.3 million families a year** — pass through roughly 6,700 hospices, and the family is known to an institution days or weeks before death. But the window is brutal: **half of hospice patients die within 19 days of enrollment, a quarter within 5 days**. The product must deliver value in 48–72 hours, not through a leisurely planning journey.

The hospice-pays B2B model has a confirmed white space: **no company today sells funeral navigation or price transparency to hospices**. The nearest analogs prove each piece separately: Help Texts sells hospices per-family grief-support subscriptions tied to the federal bereavement mandate; Empathy (\$162M raised) proved institutions will pay for family death-logistics at national scale — but its payers are life insurers and employers; Everest Funeral Concierge proved funeral price comparison works as an institutionally funded benefit — embedded in group life insurance, 25M+ covered lives. Nobody has assembled these into a hospice-funded funeral-navigation benefit.

The research also kills some assumptions. Hospice CAHPS emotional-support scores are the industry's **highest** (91% top-box nationally), so “fix your bereavement scores” is a false pitch; the honest wedge is unfunded-mandate coverage, scarce social-worker time (**~14 minutes per patient per week on average**), referral differentiation, and steering-liability relief. Consumer funeral-transparency startups have almost all died or stalled — the money has been made selling to institutions, never to grieving consumers. And revenue must come from the hospice side only: per-referral fees from funeral homes are unlawful for licensees in several states and would destroy the product's one defensible asset, neutrality.

Highest-result play: intervene in the admission week, before death — break the “first-call” default under which the funeral home that picks up the body wins by default — with verified local price and quality comparison, benefit recovery (VA burial benefits alone are unused by >75% of

eligible veteran families), and a documented plan the family controls. Measurable outcomes per family: **\$2,000–\$6,000 saved**, benefits dollars recovered, and social-worker minutes returned to clinical care.

A note on evidence: figures below were compiled and cross-checked by five parallel research passes against primary sources (NFDA GPL studies, MedPAC March 2026 report, FTC releases and Federal Register, CMS rules, DOJ filings). Where sources conflict, both numbers are shown. Where a widely quoted claim failed verification, it is flagged — several popular industry statistics did not survive scrutiny. Regulatory sections are research, not legal advice.

Part 1 — The Family Pain Map

1.1 What a funeral really costs

NFDA's 2023 General Price List study (809 funeral homes; still the latest published wave) puts the **median funeral with viewing and burial at \$8,300** and **with cremation at \$6,280**. The burial median is misleadingly low: it **excludes** the vault (median \$1,695, bringing it to \$9,995), the cemetery plot, grave opening/closing, the marker, flowers, and the obituary. Cemetery-side items typically add **\$3,500–\$10,000+**, putting a conventional burial at **\$12,000–18,000 all-in** in much of the country. Empathy's Cost of Dying report (2024) estimates the total cost of handling a death at **~\$12,600 per family**, with the funeral the largest single line (~\$5,700), plus **~420 hours** of administrative work over 12–18 months.

1.2 The same service is not the same price

This is the core exploitable inefficiency. The canonical FCA/Consumer Federation of America survey (150 homes, 10 metros) found full-service funeral prices ranging **\$2,580 to \$13,800**, with prices for identical services within a single metro varying "almost always by at least 100% and often more than 200%"; FCA/CFA state their surveys have found spreads of **up to 400–500% for the same services**. Direct cremation in a single market (e.g., Indianapolis) runs **\$845 to \$3,300+** for the identical disposition. Chain pricing is systematically worse: CFA found SCI/Dignity Memorial homes price **up to 72% above independents** in the same cities — and in the 2022 online-pricing survey, **0 of 102 Dignity Memorial locations** sampled posted prices online. FCA's California study found price-hiding funeral homes charge more than price-posting ones for identical services. Casket markups run **up to ~5x wholesale** (PIRG).

1.3 Why grieving buyers overpay

The buyer is impaired, rushed, and uninformed, and the seller knows it. In NFDA's 2025 consumer study, **54.7% of funeral planners contacted only one funeral home** — up from 47.5% the year before; a 2022 CFA/Ipsos survey found only **20% visited more than one home to compare price lists**. Arrangements are typically made within **1–2 days of death** and the funeral held within 3–7. Critically, the provider decision is usually made implicitly at the moment of death: whoever takes the "first call" and removes the body becomes the funeral home by default, and switching later means confrontation and transfer fees. Consumer knowledge is dismal: only **25%** know funeral homes must quote prices by phone (15% among those 65+), only **5%** know homes must accept an outside casket without a fee, and only **17%** know direct cremation is widely available under ~\$1,200 (CFA/FCA national survey, 2020). There is no rigorous academic study quantifying grief-driven overspending itself — the evidence base is this behavioral data plus the FTC's own duress framing — an honest gap worth knowing.

1.4 Transparency has not arrived, and regulation will not deliver it soon

The FTC Funeral Rule (1984) requires itemized price lists in person and price quotes by phone — **it does not require prices online**, and as of mid-2026 the FTC's modernization effort (ANPR November 2022, workshop September 2023) has produced **no proposed or final rule**. Only California mandates online GPL posting. The result: only **18% of 1,046 funeral homes** sampled in 35 state capitals posted prices online (CFA/FCA 2022); the FTC's own review found **>60% of funeral home websites had little or no pricing**. Even the existing rule is widely violated: FTC undercover sweeps repeatedly catch **~20–25% of homes** failing in-person disclosure duties, and the first phone sweep (2023; 250+ providers) led to 39 warning letters. The FTC's November 2024 phone-sweep staff report (278 providers) found ~50% answered

pricing questions with ranges or estimates, at least a third quoted package-only prices, and **37+ providers quoted different prices for the same service on different calls**. Meanwhile 88% of consumers now expect pricing online, and “ability to meet my budget” is the funeral profession’s single worst-rated attribute at 41% positive (Foresight Companies 2024 study, n=5,335). Strategic read: the FTC will not build the transparency layer — whoever builds and maintains a verified price database owns a scarce asset, but must build it the hard way, against seller resistance.

1.5 The financial damage

Depending on survey, **roughly 4 to 6 in 10 Americans could not pay for a funeral today without borrowing** (Debt.com 2025: 42.6% — note its own press release misstates this as 57%; CardRates 2025: 58%). Among those who took on funeral debt, 59% used credit cards; **36% of Americans say they would skip rent, utility, or credit-card payments** to cover a funeral. GoFundMe has reported **125,000+ memorial fundraisers per year raising ~\$330M** (company figures, 2018–19 vintage — dated but directionally intact). Peer-reviewed work links bereavement to lasting income and employment losses, and a Japanese study (OMEGA, 2022) found low funeral satisfaction correlates with higher post-bereavement medical and counseling costs. Funeral cost is not just a consumer-surplus problem; it compounds grief harm in exactly the population hospices are mandated to support for a year after death.

Part 2 — The Hospice Window: Why This Channel

Three structural facts make hospice the highest-leverage distribution point in all of deathcare:

- **Concentration.** 52.9% of Medicare decedents used hospice in 2024 (an all-time high) — over 1.3 million Medicare hospice deaths per year flowing through ~6,706 Medicare-certified hospices (MedPAC, March 2026). No other institution predictably touches this many pre-death families.
- **Foreknowledge.** Hospice is the only setting where death is anticipated, discussed, and staffed for. The family can be reached before the purchase — the only moment the first-call default can be broken.
- **A mandate with no product.** Hospices must provide psychosocial support and ~13 months of bereavement care, unreimbursed; staff field funeral questions with no standard tool, and are ethically constrained from recommending specific funeral homes.

2.1 The clock

Median lifetime length of stay was **19 days** in 2024; **25% of hospice decedents enroll ≤5 days before death, 10% ≤2 days**, and more than a quarter enroll only in the last week of life (MedPAC 2026). Averages (99.6 days) are inflated by long-stay outliers. Design implication: the product's core loop — understand options, see verified local prices, capture wishes, identify benefits, choose a provider — must complete in **one sitting**, with the long-runway experience an enhancement, not a requirement. Nonprofit hospices skew toward exactly the short-stay families (71-day average LOS vs 120 at for-profits).

2.2 The steering dilemma — the product's quiet superpower

Hospice social workers are expected to help with funeral planning (industry job descriptions say so explicitly), yet NHPCO's ethics code bars making referrals for anything but the patient's interest and bars compensation for referrals; consumer advocates have publicly accused hospice staff of steering families toward corporate funeral chains (US Funerals Online, 2025); and funeral homes actively court hospices — CANA literally sells funeral directors a "Certified Hospice Relations Specialist" credential for winning hospice referral relationships. So the hospice sits in an uncomfortable spot: obligated to help, forbidden to steer, and lobbied by sellers. A **neutral, no-provider-payment navigation tool is the compliant answer to the question every family asks ("who should we call?")** — it lets the hospice help without ever naming a funeral home. This is a genuine institutional pain reliever, not just a family perk.

Part 3 — The Buyer: What a Hospice Will Actually Pay For

3.1 Structure and economics

6,706 Medicare hospices (2024); **82% for-profit by count, serving ~60% of patients**; \$28.3B Medicare hospice spend (+10.4% YoY); Medicare funds ~91% of hospice days via per-diems (FY2026 routine home care: **\$230.83/day** days 1–60). Margins are the pricing constraint: aggregate Medicare margin **8.0%** (2023), for-profits **13.7%**, nonprofits **-1.3%**, and MedPAC has recommended a 0% payment update for FY2027. Consolidation is real but shallow — VITAS (ADC ~22,000) and Gentiva (ADC ~34,000) together account for under 10% of days; it is a long-tail market of small operators (average hospice: ~200 deaths/year). Read: price per family in the tens of dollars, not hundreds; land-and-expand through regional multi-site operators; expect nonprofits to be mission-receptive but budget-poor (donor/foundation funding can bridge them).

3.2 The unfunded, unmeasured mandate

42 CFR 418.64 makes bereavement counseling a Condition of Participation — an organized program available for **at least a year after death** (13 months is industry convention, not the regulation text) — with **no separate reimbursement and no CMS quality standards**. Funeral-planning help is nowhere in the CoPs, yet hospices publicly describe it as part of the social worker's job: it is unfunded, unstandardized gray-zone work. Help Texts built its entire hospice sales motion on this mandate framing ("meet the requirement, measurably, for dollars per family") — the demonstrated way to unlock a compliance-adjacent budget line.

3.3 Staff scarcity is the felt pain

The average routine-home-care hospice patient receives **~0.3 social-worker visits per week — roughly 14 minutes** (MedPAC 2026); social workers carry ~29 patients versus ~14 for nurses, and hospice has lost social workers faster than any other healthcare sector. Staffing was the #1 concern of hospice executives in both the 2025 (35%) and 2026 (40%) industry outlook surveys. Every funeral-logistics question a family asks consumes the scarcest clinical resource. A tool that absorbs the funeral conversation returns social-worker minutes to psychosocial care — quantifiable, and the pain the buyer already feels.

3.4 What hospices measure — and an assumption this research falsified

CAHPS Hospice surveys families ~2 months post-death and feeds public star ratings (only ~32% of hospices even have one; 75-survey minimum). **Falsified assumption:** bereavement/emotional support is NOT a weak spot — "emotional and spiritual support" is the highest-scoring CAHPS measure (**91% top-box nationally**). Do not pitch "fix your bereavement scores." The honest CAHPS angle is marginal differentiation on "rating 9–10" (82%) and "would recommend" (85%), flat for years, plus the referral-marketing value of a distinctive family benefit in fiercely competitive markets (California auditors found dozens of hospices in single LA buildings). For nonprofits, add donor goodwill: families who saved thousands become donors and reviewers.

3.5 Hospices already buy family-facing tools — precedent exists

Help Texts (ex-Grief Coach) sells hospices bulk 13-month grief-text subscriptions per family (retail \$99/yr; B2B pricing undisclosed), pitched on mandate coverage, CAHPS, mailer savings, and referral/donor goodwill. Compassus rolled Empathy out across **200+ locations in 30 states** after a 3-month pilot (2021). Medalogix/Muse predictive analytics runs on ~50,000 hospice

patients daily. Hospice compliance departments demonstrably approve family-facing death-logistics vendors. What none of these do is the funeral purchase itself — the largest, most painful, most time-critical line item in the family's death experience.

Part 4 — Competitive Landscape and the White Space

Sixteen companies and post-mortems reviewed. The pattern is unambiguous: **consumer-paid funeral transparency dies; institution-paid death navigation scales.**

Segment	Who	Status	Lesson
Price comparison (B2C)	Funeralocity, Parting.com, Ever Loved, Funerals360	Alive but tiny: Funeralocity ~3 employees (2026); Ever Loved <\$5M revenue; others stalled	Transparency alone doesn't monetize; grieving consumers don't pay; SEO-war acquisition
D2C cremation	Tulip, Solace, After.com, Neptune (SCI)	Tulip & Solace acquired by PE roll-up Foundation Partners (250+ locations; launched Afterall, bought Cake 2024)	Disruption got absorbed; owning supply beats brokering it
Institutional bereavement navigation	Empathy (\$162M raised, Series C May 2025); Everest Funeral Concierge (25M+ covered lives, PriceFinder)	Alive, category leaders — payers are life insurers & employers	Institutions WILL pay for family death-logistics; funeral price help works as an embedded benefit
Hospice-pays products	Help Texts (grief texts only, ~\$1M raised)	Alive, capital-light, the lone occupant	The channel works via the bereavement-mandate frame — and is uncontested for funerals
Dead/cautionary	Grace (2019), Lantern, Cake, Everdays (\$17.7M), Willing → MetLife, Farewill (£30M raised, sold \$16.8M)	Dead, absorbed, or exited small	One-time purchase, grief-window conversion, thin margins; exits go to insurers/incumbents

The Grace warning. Grace, a cremation startup, used hospices and hospitals as lead-gen — including a hospital EHR feed that auto-texted the company when a patient “Expired” — and its founder noted hospice enrollment “typically meant a conversion within 24 days.” It shut down in 2019, but it is exactly what hospices fear from funeral-adjacent vendors. Honest Funeral must be architecturally incapable of this: no provider payments, no lead selling, family-initiated contact only. The channel is scarred; neutrality is the entry ticket and the brand.

White-space conclusion (verified): extensive searching found **no company selling funeral planning or price transparency to hospices as the payer.** The commercial relationships all run the other direction — funeral homes paying to court hospices for referrals. The institutional-payer model is proven (Empathy, Everest), the hospice-pays model is proven (Help Texts), and their intersection — hospice-funded funeral navigation — is empty.

Part 5 — Where Honest Funeral Produces the Highest Result

Ranked by pain severity × frequency × measurability of outcome × buyer willingness to pay × competitive vacuum:

#	Opportunity	Measurable result	Frequency	Competition
1	Pre-death navigation in the admission week: break the first-call default with verified local price/quality comparison and a documented family plan	\$2,000–\$6,000 saved per family; % families with 3+ quotes before death; decision made calmly pre-death	Every hospice death (1.3M+/yr Medicare alone); usable window in ~75% of cases	None in channel
2	Verified price layer: maintained, verified GPL database for the hospice's service area	Quote accuracy vs. FTC-documented phone chaos; the defensible data asset	Reused across every family in a metro	Weak, stagnant B2C players; sellers resist — moat if solved
3	Staff relief + compliance wedge (the sales pitch): absorb the funeral conversation; give social workers a neutral, compliant answer to “who should we call?”	Social-worker minutes returned; steering-liability relief; bereavement-mandate coverage story	Daily staff pain at every hospice	Help Texts adjacent (grief only)
4	Benefit recovery: VA burial benefits (<25% of eligible veteran families use them; ~2/3 don't know they exist), SSA \$255, state/county indigent programs, life-insurance checks	Dollars recovered per family (FEMA's COVID program averaged ~\$6,500 when navigation existed); equity story for nonprofit boards	Subset of families, high per-hit value	Empathy touches this for insurer clients; nobody in hospice channel
5	13-month bereavement companion: post-death admin checklist, grief resources, memorial tools	Contract expansion, CAHPS-window presence, donor pipeline for nonprofits	Every death, long tail	Help Texts, Empathy — crowded; do later, not first

5.1 Why #1 is the wedge

Every dollar of family value concentrates at one moment: before the first call. After death, the provider decision closes in 24–72 hours and ~55–80% of families never get a second quote. Before death, the family can compare calmly — and the spread between the low- and high-priced provider for the identical service in one metro is thousands of dollars. The hospice admission packet / social-worker intro is the only scaled, trusted delivery mechanism that reaches families at that moment. Cremation's rise (63.4% in 2025 → ~82% by 2045) makes the comparison even cleaner: direct cremation is a near-commodity where price variance is pure information failure (\$845–\$3,300 in one market).

5.2 What to say to the hospice (and what not to)

- **Lead with:** unfunded mandate coverage; social-worker time returned; a compliant answer to referral questions; family financial protection as differentiator in referral markets; nonprofit angles (donor goodwill, mission fit).
- **Do not lead with:** “improve your emotional-support CAHPS scores” (they’re already 91%); anything implying the hospice steers families to cheap providers (positioning is informed choice, not cheapness — some families rationally choose the \$12,000 service, now knowingly).
- **Instrument from day one:** median \$ saved vs. metro median price, quotes per family, benefit dollars recovered, staff minutes saved (survey), family NPS, and — over time — CAHPS recommend delta at pilot sites. No RCT links funeral pre-arrangement to grief outcomes yet; a pilot that publishes first-of-kind outcome data owns the category’s evidence base.

5.3 What NOT to build first

Not a consumer app or SEO site (the graveyard), not a pre-need payment product (prepaid-funeral fraud is a documented disaster — \$450M NPS collapse; position as “plan and document everything, prepay only with portable, insured instruments”), not a funeral-home-paid marketplace (illegal referral fees in NY/VA and elsewhere for licensees; destroys neutrality), and not grief support (crowded, and it isn’t the money moment).

Part 6 — Business Model Implications

Hospice pays, per family served — the Help Texts pattern — is the model this research supports. Sizing anchors (illustrative, not market-quoted: B2B price points in this category are negotiated and undisclosed):

- Average hospice ≈ 200 deaths/year. At \$25–\$75 per family, a typical contract is **\$5,000–\$15,000/year** — below most hospices' signature thresholds for a mandate-adjacent, staff-saving tool, and priced within the for-profit margin cushion (13.7%). Full Medicare-hospice penetration at \$50/family ≈ **\$65–70M ARR ceiling** on the wedge product alone — a real but not enormous market, arguing for expansion paths (bereavement companion, non-hospice channels: palliative programs, health systems, payers, employers/insurers à la Empathy/Everest) once the wedge proves out.
- Chains change the math: VITAS alone (~22,000 average daily census) and Gentiva (~34,000) each represent tens of thousands of deaths/year — single contracts worth \$1M+ at per-family pricing, but slow enterprise sales. Land with regional 2–20-site operators and mission-driven nonprofits; use published pilot metrics to open chains.
- **Never take money from funeral homes per referral.** NY Pub. Health Law §3450 and Virginia's anti-cappers statute make paying for funeral patronage a license-revocation offense for the funeral home; broad state definitions of unlicensed "funeral directing" (TX, SC) mean the platform must stay on the education/navigation side of "arranging"; and Grace showed how provider-paid models poison the hospice channel. If the supply side is ever monetized, only flat, disclosed, non-contingent listing/SaaS fees — but the clean version is hospice-pays only, and "we take nothing from funeral homes" is the marketing.
- Nonprofit hospices (–1.3% margins) are mission-aligned but broke: offer foundation/donor-funded sponsorships or tiered pricing; their family populations (shorter stays) need the product most.

Part 7 — Risk Map and Non-Negotiable Design Rules

Risk	What the research shows	Design rule
Anti-kickback (AKS/CMP)	Funerals aren't federally payable, so funeral referrals themselves fall outside AKS/CMP — but a free family benefit used to induce HOSPICE selection is the danger vector (one-purpose test); OIG guidance says benefits disclosed only after provider selection don't violate the inducement bar	Deliver post-admission only; keep the product out of hospice pre-admission marketing; frame as bereavement/psychosocial support procurement
State funeral law	Referral-fee bans (NY, VA); broad “funeral directing” definitions (TX; SC bans aiding unlicensed practice); at-need solicitation bans (FL, TX, ME, NE) restrict outbound contact with grieving families	No per-referral revenue; navigation/education positioning, never “arranging”; family-initiated activation only — the hospice hands it to the family, the platform never cold-contacts next of kin
Privacy (HIPAA)	Decedent PHI is protected 50 years; receiving patient/family data from a hospice makes the platform a business associate	Sign BAAs; or design family-self-serve activation so the hospice transmits nothing
Hospice compliance climate	Historic scrutiny: Special Focus Program (paused Feb 2025), June 2026 DOJ takedown included a hospice owner buying decedent data from a funeral-home employee, and a nationwide 6-month moratorium on Medicare enrollment of NEW hospice providers from May 13, 2026 (existing hospices and patient elections unaffected)	Expect slow, conservative vendor diligence; lead with compliance documentation; the Empathy/NHPCO and Help Texts precedents show the category clears review when there are no steering payments
Price-data acquisition	Only ~18% of homes post prices online; phone quotes are inconsistent by the FTC's own measurement; NFDA formally opposes mandatory online posting; no FTC online rule coming soon	Budget for manual GPL collection (Funeral Rule guarantees in-person GPL access); verify and date-stamp quotes; treat the database as the moat — no documented C&D/litigation pattern against aggregators exists, but expect friction
FTC/UDAP exposure	FTC sued online cremation broker Legacy Cremation Services for misrepresenting locations/prices — online funeral intermediaries are on the FTC's radar	Rigorous accuracy claims; show data vintage; never misstate affiliation with providers
Channel trust	Grace burned the hospice channel with lead-gen; consumer advocates already allege hospice steering to chains	Radical, auditable neutrality: publish the no-payment policy, methodology, and data practices

Part 8 — Hard Truths (Assumptions This Research Falsified or Failed to Verify)

- **“Hospice bereavement scores are weak” — false.** Emotional support is CAHPS's highest-scoring measure (91%). Sell staff time and mandate coverage, not score repair.
- **“People will comparison-shop if you give them a tool” — unproven and trending the wrong way.** Single-home shopping rose to 54.7% in 2025. The product must do the comparing for the family, pre-death, through a trusted intermediary — not hope grieving people become bargain hunters.
- **“Pre-planning saves 20–40%” — unverifiable.** That figure traces to pre-need marketing, not any published study. Use the verified dispersion data (100–200%+ within metros) and CFA's “comparison shopping can cut costs up to 50%” instead.
- **“57% of Americans can't afford a funeral” — misquoted.** Debt.com's own table says 42.6%; a separate survey says 58% would need to borrow. Cite the range, or the claim collapses under diligence.
- **“The FTC is about to force prices online” — no.** Nothing has moved past a 2022 ANPR and a 2023 workshop as of mid-2026.
- **“Grief tech is a big VC category, so consumers will pay” — the graveyard says otherwise.** Every scaled winner is institution-paid. The hospice-pays choice is the right one; the open question is contract value, not model validity.
- **Known evidence gaps:** no national data on what share of hospice families pre-arrange funerals; no RCT tying funeral pre-arrangement to grief outcomes (nearest analog: the Detering advance-care-planning RCT — family stress median 5 vs 15, anxiety and depression significantly lower); B2B price points in the category are undisclosed. A well-instrumented pilot generates the category's first proprietary outcome data — an asset in itself.

Part 9 — 90-Day Validation Plan

- **Weeks 1–4:** 20 discovery interviews — hospice social workers, bereavement coordinators, and 2–3 hospice executives (one regional for-profit, one nonprofit). Test: who owns the budget, what triggers the funeral conversation today, reaction to per-family pricing, compliance objections. Simultaneously hand-build a verified GPL database for ONE metro (Funeral Rule guarantees in-person GPL access).
- **Weeks 5–8:** paper-pilot with 1–2 hospices: a concierge version (human + spreadsheet) serving 10–20 families. Measure the five core metrics (\$ saved vs. metro median, quotes per family, benefits recovered, staff minutes, family NPS).
- **Weeks 9–12:** convert results into the sales narrative; test pricing (\$25/\$50/\$75 per family) with 5 more hospices; decide build scope. Gate to proceed: hospices sign paid LOIs AND families in the pilot saved a median $\geq$ \$1,500 or recovered equivalent benefits.

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Hospice industry (buyer)

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- NHPCO Facts & Figures 2024 (CY2022 data) — https://allianceforcareathome.org/wp-content/uploads/2024/09/Facts-Figures-2024_FINAL.pdf
- 42 CFR 418.64 — core services incl. bereavement counseling — <https://www.ecfr.gov/current/title-42/chapter-IV/subchapter-B/part-418/subpart-C/subject-group-ECFR35b48a647589673/section-418.64>
- CMS FY2026 hospice payment rates — <https://www.cms.gov/newsroom/fact-sheets/fy-2026-hospice-wage-index-and-payment-rate-update-and-hospice-quality-reporting-program>
- Hospice News — bereavement unreimbursed / consensus-standards bill — <https://hospicenews.com/2022/08/11/federal-bill-calls-for-bereavement-care-consensus-standards/>
- Hospice News — social worker caseloads and attrition — <https://hospicenews.com/2023/05/26/3-keys-to-hospice-social-worker-recruitment-and-retention/>
- Hospice News — 68% of hospices lack star ratings (2025) — <https://hospicenews.com/2025/06/18/68-of-hospices-lack-star-ratings/>
- CAHPS Hospice star ratings methodology — <https://hospicecahpsurvey.org/en/public-reporting/star-ratings/>
- US Funerals Online — hospice steering toward corporate funeral homes (2025) — <https://www.us-funerals.com/the-rising-cost-of-farewell-why-hospice-staff-may-be-steering-families-toward-corporate-funeral-homes-and-what-families-can-do-about-it/>

Competitors and analogs

- Empathy \$72M Series C (Fortune, May 2025) — <https://fortune.com/2025/05/29/exclusive-empathy-raises-72-million-series-c-to-tackle-the-agonizing-logistics-of-death/>
- Compassus × Empathy rollout (Hospice News, 2021) — <https://hospicenews.com/2021/07/14/compassus-partners-with-tech-company-empathy-on-bereavement-support/>
- Everest Funeral Concierge — employer/group-life funeral benefit — <https://everestfuneral.com/group-life/>
- Help Texts for hospices (hospice-pays grief support) — <https://helptexts.com/for-organizations/hospices/>
- Grief Coach → Help Texts (Hospice News, 2023) — <https://hospicenews.com/2023/01/09/grief-coach-rebrands-to-help-texts-launches-health-care-worker-support-service/>
- Foundation Partners launches Afterall, acquires Cake (2024) — https://www.kates-boylston.com/industry_news/foundation-partners-group-launches-afterall-to-guide-families-through-the-end-of-life-experience/article_c39e812c-80b0-11ef-a25a-371bbd28d791.html
- Grace founder post-mortem (hospice channel lead-gen) — <https://krugerwrites.com/p/that-time-i-created-a-2m-funeral>
- Farewill sold for \$16.8M after £30M+ raised (TechCrunch, 2024) — <https://techcrunch.com/2024/10/17/uk-digital-end-of-life-services-startup-farewill-acquired-for-16-8m/>
- Everdays shutdown chronicle (Coverager) — <https://coverager.com/everdays-site-goes-dark/>
- CANA Certified Hospice Relations Specialist (funeral homes courting hospices) — <https://www.cremationassociation.org/chrs.html>

Regulatory and risk

- OIG fraud & abuse laws overview (AKS, CMP scope) — <https://oig.hhs.gov/compliance/physician-education/fraud-abuse-laws/>
- Holland & Hart — patient inducements: law and limits — <https://www.hollandhart.com/patient-inducements-law-and-limits>
- DOJ June 2026 national takedown (incl. hospice–funeral home decedent-data scheme) — <https://www.justice.gov/opa/pr/national-health-care-fraud-takedown-results-455-defendants-charged-connection-over-65>
- CMS nationwide moratorium on new hospice provider enrollments (Federal Register, May 2026) — <https://www.federalregister.gov/documents/2026/05/15/2026-09718/medicare-medicaid-and-childrens-health-insurance-programs-announcement-of-nationwide-temporary>
- NY Public Health Law §3450 (funeral referral-fee ban) — <https://www.nysenate.gov/legislation/laws/PBH/3450>

- Virginia funeral practice law (anti-cappers/steerers) — <https://law.lis.virginia.gov/vacodefull/title54.1/chapter28/>
- Texas Occupations Code ch. 651 (funeral directing definition; solicitation) — <https://statutes.capitol.texas.gov/Docs/OC/htm/OC.651.htm>
- Florida §497.164 (at-need solicitation ban) — <https://law.justia.com/codes/florida/title-xxxiii/chapter-497/part-i/section-497-164/>
- St. Joseph Abbey v. Castille, 712 F.3d 215 (5th Cir. 2013) — <https://ij.org/case/saint-joseph-abbey-et-al-v-castille-et-al/>
- HHS — HIPAA and decedent information (50-year rule) — <https://www.hhs.gov/hipaa/for-professionals/privacy/guidance/health-information-of-deceased-individuals/index.html>

Outcomes and benefits

- Detering et al., advance care planning RCT (BMJ 2010) — <https://pubmed.ncbi.nlm.nih.gov/20332506/>
- Becker et al., funeral satisfaction and post-bereavement costs (OMEGA 2022) — <https://journals.sagepub.com/doi/10.1177/0030222820947573>
- FEMA COVID-19 Funeral Assistance (\$3.26B, ~506K awards) — <https://www.fema.gov/disaster/historic/coronavirus/economic/funeral-assistance>
- VA burial benefits underuse (MOAA/Stars & Stripes, 2024) — <https://www.moaa.org/content/publications-and-media/news-articles/2024-news-articles/health-care-and-earned-benefits/va-aims-to-emphasize-free-burial-benefits-for-veterans-at-national-cemeteries/>
- Social Security \$255 death benefit, unchanged since 1954 (CNBC, 2024) — <https://www.cnbc.com/2024/09/26/social-security-death-benefit-has-been-255-since-1954-that-may-change.html>
- NACo — indigent burials straining county budgets — <https://www.naco.org/articles/undertakers-last-resort-indigent-burials-rise-denting-county-budgets>
- DOJ — National Prearranged Services pre-need fraud (~\$450M) — <https://www.justice.gov/usao-edmo/pr/former-president-national-prearranged-services-inc-and-ceo-lincoln-memorial-life>